Reducing Chronic Absenteeism in Washington, D.C. Public Schools:
A Comprehensive Policy Proposal for Mayoral Action

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I. INTRODUCTION & BACKGROUND

Chronic absenteeism (CA), defined as a student missing 10% or more of school days within an academic year, is one of the most critical issues facing the D.C. public education system. In the 2024 to 2025 academic year, about 4 in 10 public-school students in K to 12th grade were chronically absent (OSSE, 2025), raising significant concerns about their long-term academic performance, economic outcomes and socioemotional development, among other potential impacts.

Washington, D.C. is uniquely positioned to address CA through strong mayoral authority. Under the 2007 Public Education Reform Amendment Act, the mayor controls all aspects of D.C. Public Schools, with authority over curricula, operations, budgets, staffing, and facilities, allowing for swift, comprehensive interventions (D.C. Official Code § 38-172(b), 2007).

This paper examines the trends, consequences and root causes of CA in D.C., with particular attention to the disparities that have deepened in the aftermath of the COVID-19 pandemic. Based on the key determinants of absenteeism, policy gaps, and evidence-based practices in other urban school districts, we propose that the mayor adopt a comprehensive five-pillar framework centered on family engagement, early warning systems, community hubs, school-based health services, and accountability mechanisms. To make these recommendations as actionable as possible, we proceed by outlining our implementation plan, funding strategy, and evaluation of political dynamics and stakeholders. Through this analysis, we aim to identify and enable the adoption of effective, efficient and equitable policies to significantly reduce chronic absenteeism and support at-risk student populations.

II. PROBLEM ANALYSIS

A. Trends and Disparities in D.C.

Washington D.C. is facing a chronic absenteeism (CA) crisis. 39.5% of K-12 students were chronically absent in 2024-25, compared to the national rate of 22% (OSSE, 2024; OSSE, 2025; Diliberti et. al, 2025). While CA has steadily decreased since the height of the pandemic in the 2021-22 academic

year, the current rate continues to substantially exceed pre-pandemic levels and has plateaued since 2023-24 (see Appendix Figure 1), suggesting the lingering impact that COVID-19 has had on student attendance.

Because CA in D.C.'s public schools is concentrated at the high school level (56%), and highest in the ninth grade (65.25%), the transition into high school is a key time for intervention (Coffin & Mason, 2025). Students in the public school system are more likely to be absent than their charter school peers (ibid). Moreover, CA disproportionately impacts marginalized populations, including students who are economically disadvantaged (45.2%), homelessness (41.8%), or under the care of D.C's Child and Family Services Agency (60.5%); all such students are considerably more absent from school than their non-marginalized counterparts (OSSE, 2024). Wards 7 and 8, home to the highest concentrations of students living in poverty, have the highest CA rates (Office of the Deputy Mayor for Education, n.d.). There are also significant racial disparities: Black students are 3.9 times more likely, and Hispanic students 2.8 times more likely, to be chronically absent than their White peers (OSSE, 2024).

Post-pandemic data analysis reveals shifts in CA patterns. Eighth grade saw the largest increase in CA, by 21 percentage points (Coffin and Mason, 2025). Female students are now more likely to be chronically absent, a reverse of pre-COVID patterns. In high schools, absenteeism is tied less to student behavior and more to individual or family circumstances (ibid). These trends suggest a need for greater student and parent-based interventions in middle school with particular attention to female students.

B. Root Causes of Chronic Absenteeism

Absenteeism stems from interrelated poverty, health, family, school, and broader community factors, suggesting that successful policies for mitigating CA must address multiple causes simultaneously (see Causal Model in Appendix Figure 2). Economic disadvantage is one of the most powerful predictors of future absenteeism in D.C. (Coffin and Mason, 2025). Low-income students often face attendance barriers such as housing instability, food insecurity, and unreliable transportation (Humm Patnode et al., 2018; Ready, 2010; Gubbels et al., 2019). Chronic physical and mental health conditions, unsafe school climates, bullying, exclusionary discipline, and unstable family dynamics (e.g., low

parental involvement) further reduce attendance (Lum et al., 2017; Finning et al., 2019; Van Eck et al., 2017; Laith & Vaillancourt, 2022; Kearney, C. A., & Graczyk, 2014; Learning Heroes & TNTP, 2023). At the community level, neighborhood violence and segregation create additional attendance barriers (Opara et al., 2022; Berman et al., 2018; Palardy et al., 2015).

These patterns are present in D.C., with post-pandemic listening sessions revealing significant disengagement and perceptual obstacles, including mental health difficulties and a feeling that in-person learning is less essential (Mason, 2024). Students and families also commonly cite transportation safety concerns and illness as barriers to attendance (Coffin & Mason, 2025).

Perhaps the most notable facet of CA is its persistence. Students chronically absent in pre-K are 41.5% more likely to be chronically absent in kindergarten (OSSE, 2024). This holds in later grades, as well: 8th grade absenteeism is the strongest predictor of 9th grade chronic absenteeism (Coffin & Mason, 2025), underscoring the importance of early warning systems, proactive communication, and outreach to moderately absent students before patterns solidify.

C. Consequences of Chronic Absenteeism

CA carries academic, social, and economic consequences. Students who miss substantial instructional time score lower on standardized tests and are more likely to repeat a grade (Santibañez & Guarino, 2021; Humm Patnode et al., 2018). Absenteeism also disrupts classroom instruction and peers' learning while undermining socioemotional development, including belonging and motivation (ibid; Gottfried, 2015). Long-term, chronically absent students are less likely to graduate high school or complete postsecondary education and face reduced lifetime earnings and weaker civic engagement (Ansari et al., 2020; Jacob & Lovett, 2017). For schools, widespread absenteeism increases remediation needs, strains staffing, and can damage community trust (Lieberman, 2024).

D. Current Policy Landscape

In the post-pandemic period, D.C. has prioritized absenteeism reduction. The Bowser administration has committed to reducing chronic absenteeism by 50% in 5 years (Attendance Works, n.d.). Efforts are spearheaded by a new "Every Day Counts!" taskforce, which has created an attendance

website resource hub, launched a public messaging campaign, and began an attendance pledge drive. Policymakers are also reshaping the legal framework to emphasize supportive over punitive responses. The 2024 Truancy Reduction for Student Success Act created a pilot program to refer students with 15 unexcused absences to human services rather than the juvenile probation agency (Council of the District of Columbia, 2024). In its first year, participating students showed a 50% increase in attendance and improved behavioral functioning; the pilot is now being extended and expanded for a second year (D.C. Department of Human Services, 2025). Furthering these efforts, Councilmember Parker has introduced the Improving School Attendance Outcomes Amendment Act of 2025, which would strengthen early intervention efforts based on the pilot's findings.

D.C. has also increased its support for programs targeting core attendance barriers. The Show Up, Stand Out program creates individualized attendance plans alongside parents of K-8th grade children with five or more absences (Office of Victim Services and Justice Grants, 2021). Kids Ride Free and Safe Passage address transportation cost and safety barriers (District Department of Transportation, n.d.; DC.Gov, n.d.). In addition, local partnerships, such as the one with Children's National Hospital, address health barriers to educational attendance (Children's National Hospital, n.d.).

Despite these investments, significant gaps remain. Data collection and reporting remain fragmented across agencies (AJE-DC, 2025). Stronger coordination amongst CFSA, DHS, OSSE, and DCPS is necessary to generate more timely interventions. Moreover, a lack of coordination between families, schools, healthcare providers, nonprofits and government agencies leaves vulnerable students without a holistic support system (ibid). Resource constraints further limit intervention capacity. These challenges point to the need for collaborative, data-driven, and transparent policy strategies.

III. EVIDENCE OF EFFECTIVE INTERVENTIONS

Research from other major urban school districts reveals several evidence-based interventions that have successfully reduced CA and could inform D.C.'s strategy.

A. Family and Community Engagement Models

Urban districts and states are increasingly investing in family and community partnerships to reduce chronic absenteeism, with growing evidence of their effectiveness. A longitudinal study of 39 schools found that partnership practices, such as clear communication of attendance expectations, resource provision, and community engagement, significantly reduced absenteeism, even after controlling for school-level factors (Sheldon & Epstein, 2004).

Recent initiatives build on these insights: Chicago's Family and Community Engagement (FACE) program transforms schools into resource hubs offering multilingual workshops, social service referrals, and culturally responsive programming to build trust with families and address root causes such as housing instability and lack of mental health care (Chicago Public Schools, 2025); Rhode Island's cross-sector strategy unites educators, civic leaders, and health professionals around a public "Student Attendance Leaderboard" and tailored outreach plans, yielding measurable improvements in student attendance (Miller et al., 2024; Cohen, 2024); and Connecticut's Learner Engagement and Attendance Program (LEAP) has improved K–5 attendance by 8 percentage points and grades 6–12 by 16 percentage points through $10 million in state funding for intensive home visits (Miller et al., 2024).

The Community Schools Model (CSM) has also emerged as a central strategy for addressing CA by embedding supports (e.g., mental health care, tutoring, family engagement, and after-school enrichment) within school environments, thereby eliminating many of the logistical and economic barriers that drive student disengagement. An independent evaluation conducted by the RAND Corporation found that schools implementing the CSM demonstrated improvements in school climate, academic expectations, and relationships among staff, students, and families, all of which are closely associated with higher attendance rates (Johnston et al., 2017). These findings are consistent with those of Maier et al. (2017), who reviewed over 140 studies on community school implementation nationwide and concluded that such models are particularly effective in improving attendance and student engagement when implemented with strong partnerships and sustained support.

B. Low-Cost Behavioral Nudges

Rogers and Feller (2016) demonstrated the power of low-cost behavioral nudges in a Philadelphia field experiment with over 40,000 students, where postcards highlighting comparative attendance data reduced total absences by 6 percent and chronic absenteeism by 11 percent at a cost of just $6 per additional day of attendance. Similar innovations include Pittsburgh's two-way texting pilot, which allowed staff to send personalized attendance updates while enabling families to respond with questions or share logistical barriers, such as transportation or clothing needs (Smythe-Leistico & Page, 2018). Praised for its immediacy, affordability, and relational tone, the texting program fostered collaboration and trust between families and schools (McArdle, 2024). While not sufficient alone, these behavioral strategies offer strong returns on investment and can serve as gateways to deeper interventions, complementing mentorship, school climate initiatives, and broader attendance supports.

C. School-Based Health Centers (SBHCs)

These are on-site clinics that provide primary medical care, mental health services, and preventive screenings directly within school settings, often in partnership with local health providers. By eliminating barriers such as cost, transportation, and stigma, SBHCs increase access to care for underserved students and allow health needs to be addressed without students missing class time. A recent large-scale study of over 14,000 students across 17 SBHCs found that attendance trajectories improved after students' first visit, reversing prior declines, with the strongest gains observed among those receiving mental health care (Lim et al., 2023). For these students, attendance rose significantly after their initial mental health encounter, suggesting SBHCs play a particularly critical role in mitigating absenteeism linked to behavioral health challenges. Beyond attendance, SBHCs contribute to educational equity by strengthening school connectedness, addressing chronic conditions, and supporting at-risk populations who might otherwise go without care. These findings underscore the value of sustained investment in SBHCs as both a public health and educational strategy.

D. Data-Driven Interventions: Early Warning and Multi-Tiered Systems

New York City and Oakland Unified School District illustrate how early warning systems (EWS) and multi-tiered approaches can reduce chronic absenteeism through proactive, data-driven interventions.

In NYC, EWS integrates real-time data on attendance, academics, and behavior into school workflows, with the DOE providing weekly reports that flag students missing five or ten days of school (REL Southwest, 2025; Hollie, 2024). Schools use this data within collaborative team structures and tiered frameworks, such as Gotham Collaborative High School's four-level intervention model, which ranges from peer support to wraparound case management. The city's partnership with Branching Minds has further enabled systemwide implementation of multi-tiered systems of support (MTSS), allowing educators to track risk patterns, document interventions, and coordinate responses (Branching Minds, 2025; McMahon & Sembiante, 2020). Similarly, Oakland Unified reduced chronic absence from 16 percent to 11.9 percent between 2005–06 and 2013–14 by combining data analysis, early interventions, family engagement, and community partnerships, with schools like Garfield Elementary and Roosevelt Middle demonstrating particular success (Attendance Works, 2019). Together, these cases highlight the effectiveness of coordinated, preventive strategies that pair robust data systems with personalized supports.

IV. POLICY PROPOSAL

To address the root causes of chronic absenteeism and the shortcomings of current policies aimed at reducing it, we propose that the mayor adopt a data-driven, equity-centered, interagency strategy built on five key pillars.

1. Tiered Family Engagement Corps. A citywide team of trained, culturally responsive liaisons would be deployed to high-absence schools to conduct outreach, coordinate supports, and bridge linguistic and cultural gaps. The model builds on Chicago's FACE initiative, Connecticut's LEAP program, and Pittsburgh's two-way texting pilot.

2. Standardized Early Warning Systems (EWS) and Multi-Tiered Systems of Support (MTSS). A districtwide EWS would use real-time data on attendance, academics, and behavior to flag at-risk students, paired with MTSS to integrate this data, track risk patterns, document interventions, and guide collaborative support. Together, these tools would give schools a consistent data infrastructure to act on early warning signs in a coordinated way.

3. Community-Based Attendance Action Hub. Staffed by cross-agency teams (DHS, CFSA, DHCD, DCPS, and CBOs), these hubs would coordinate with school liaisons and provide walk-in access to services addressing barriers such as housing insecurity, food access, childcare, and transportation. This system would complement the success of the D.C. Public School Connected Schools Model, in which 18 schools provide health and social services, youth and community development, and academic enrichment to their surrounding community. While this model provides a promising start, it does not adequately address the needs of students in all 117 public schools. The proposed action hubs—one in each ward—will cohere currently fragmented government services to empower students and families. Rather than duplicating existing supports, the action hubs would serve as community-level integrators to close coordination gaps and strengthen referral pathways.

4. Integrated School-Based Health Centers (SBHCs) and Mobile Health Units. Currently, D.C. Health operates seven SBHCs, an important but insufficient network given the scale of the absenteeism crisis. To address this gap, the District should expand SBHCs to all schools with chronic absenteeism above 30% and deploy mobile health units to campuses without on-site clinics, ensuring access to physical and behavioral health care regardless of ability to pay.

5. Student Attendance Leaderboard. Modeled after Rhode Island's statewide strategy to reduce CA, this tool would highlight schools making progress or equity-driven gains in attendance, motivating continuous improvement and promoting the sharing of best practices across schools.

V. IMPLEMENTATION AND EVALUATION PLAN

Rolling out this strategy will require a phased, adaptive approach that builds on existing infrastructure while addressing coordination gaps across D.C.'s education and human services systems. Implementation should begin with a three-year pilot phase targeting the 12 schools with the highest chronic absenteeism rates, approximately 10% of D.C.'s 117 public schools. These schools, largely concentrated in Wards 7 and 8, will serve as proof-of-concept sites to refine interventions before citywide expansion.

The Office of the State Superintendent of Education (OSSE) will establish an Office of Equitable Attendance (OEA) to coordinate all five pillars across agencies and schools. The OEA would be responsible for: (1) administering funding streams and interagency memoranda of understanding (MOUs); (2) overseeing implementation partners such as the Department of Human Services (DHS), Child and Family Services Agency (CFSA), Department of Housing and Community Development (DHCD), and Department of Behavioral Health (DBH); and (3) providing schools with technical assistance, data management tools, and guidance on equitable practices.

During the first year, the OEA will conduct baseline assessments to map existing attendance initiatives, identify overlapping responsibilities, and align community partners. Concurrently, OSSE will launch training modules for school-based staff and family liaisons. Each pilot school will designate an Attendance Leadership Team (ALT), composed of administrators, counselors, and community representatives, to coordinate interventions using real-time data from EWS and MTSS.

The second and third years will focus on scaling and refinement. Attendance Action Hubs will be established in each ward to connect families to services such as transportation vouchers, food access, and housing assistance. OSSE will also expand access to School-Based Health Centers (SBHCs) and deploy mobile health units to campuses without onsite facilities.

A key function of the OEA will be to implement a continuous improvement cycle, using quantitative and qualitative data to evaluate effectiveness. Schools will report monthly attendance trends through OSSE's longitudinal data system, which will feed into the public-facing "Attendance Leaderboard." This platform will track progress by school and demographic subgroup, promote transparency, and encourage healthy competition across campuses.

Evaluation will occur through three complementary measures:

1. **Quantitative Outcomes:** Changes in chronic absenteeism rates, disaggregated by grade level, race, and socioeconomic status, will be tracked quarterly. Benchmarks will be set to achieve a 10 percentage point reduction in pilot schools by Year 3.

2. **Process Metrics:** Implementation fidelity will be assessed through participation logs (e.g., number of family home visits, hub referrals, and health visits). Data will also capture response times to EWS alerts and engagement rates from families receiving outreach.
3. **Qualitative Evaluation:** Annual stakeholder focus groups, including students, parents, teachers, and community partners, will assess perceived barriers, cultural responsiveness, and trust in the attendance system.

Findings from the pilot phase will inform a districtwide expansion beginning in Year 4, guided by lessons learned and cost-effectiveness analyses. The evaluation process will also include an external review by an independent research partner, such as the D.C. Policy Center or a local university, to ensure rigor, transparency, and replicability.

VI. COST ANALYSIS AND FUNDING CONSIDERATIONS

Funding Sources

Four unique funding sources have been identified for the following budget: the Every Student Succeeds Act (ESSA), Temporary Assistance for Needy Families (TANF), Medicaid Reimbursement, and grants from the Health Resources and Services Administration (HRSA).

ESSA provides grants to schools serving "at-risk students" who are in danger of academic failure. Specifically, Titles I-A ($53,304,021 pool for attendance improvement and interventions for at-risk students), II-A ($10,295,324 pool for professional development, staff recruitment, retention, and training), and IV-A ($6,358,350 pool for well-rounded education, safe and healthy schools, and student support programs) can be applied to this policy's budget items (OSSE, 2022).

Additionally, DC can draw from a TANF pool dedicated to District-wide initiatives ($3,354,561) whose funds are designed to support programs and services that benefit low-income families across the District (DCPS, 2024).

DC can also access Medicaid Reimbursement schemes to offset up to $16,200,000 in eligible health-related program expenses (DCPS, 2024).

Lastly, remaining funding gaps can be closed with grant funding from the HRSA. The average grant value is $4,737,990.81 (HRSA, 2025).

Expenditures in the First Five Years

1. Family Engagement Corps (Subtotal: $2,873,510.91). $2,810,318.23 from ESSA Title I-A will be required to employ one social worker full-time in each ward to run family outreach (DCPS, 2025). An additional $59,832.68 from ESSA Title I-A will be spent on one-time onboarding and approach-specific training for each of these social workers, operationalized as 4 weeks of training prior to each worker's first school year. $3,360 from ESSA Title I-A will be dedicated to needs regarding communications and technology needed to effectively administer the Corps (BambooHR, 2019).

2. Early Warning Systems (Subtotal: $1,637,012.45). The financial cost for staffing the EWS and MTSS systems will be $0 outside of training, as the Superintendent can deputize existing staff such as counselors and vice principals. There will be one such "data steward" assigned per 10 schools. The training of these data stewards will total $92,012.45 and will be drawn from ESSA Title II-A. $1,545,000 from ESSA Title I-A will be dedicated for EWS and MTSS platforms.

3. Attendance Action Hubs (Subtotal: $5,991,759.66). $2,810,318.23 split between TANF and ESSA Title IV-A represents hiring one full-time coordinator per hub. An additional $2810318.23 will be spent compensating partner agencies (DHS, CFSA, DHCD, and DCPS) for caseworkers deployed to hubs so as to account for potential staffing shortfalls or overtime pay. $337,523.20 split between TANF and ESSA Title I-A will be reallocated to DC Public Libraries for physically hosting each hub in community space, compensated at a rate ¼ of current office leasing rates (Brexton Commercial, 2025). Lastly, $33,600 will be set aside for communications materials, representing thousands of physical copies per agency per ward each year (VistaPrint, 2025).

4. Mobile Health Units (Subtotal: $13,240,127). With two mobile health units deployed per ward, $12,270,914.86 represents full-time staffing by one nurse practitioner and one behavioral social worker per unit (Indeed, 2025). The vehicle fleet itself will cost $800,000 in HRSA grants (Infinity Chassis, 2024). An additional $49,546.84 from Medicaid Reimbursement will stock each unit with

adequate medical supplies (Wang et al., 2014). To unlock certain portions of that Medicaid Reimbursement stream, social workers will be trained in disability support, costing $119,665.36 in ESSA Title IV-A funds.

5. Attendance Leaderboard (Subtotal: $25,000.00). Given existing data and web infrastructure and those added in the above pillars, it will only cost $25,000 to administer the leaderboard and produce an annual report highlighting top performers and best practices.

Office of Equitable Attendance (Subtotal: $2,913,081.82). The OEA will hire three permanent staff in year one, totaling $2,600,201.82 from TANF and ESSA Title IV-A after five years (DCPS, 2023). An additional slate of interns and fellows will be phased in over five years, amounting to $152,800 from ESSA Title I-A (DOES, 2022). $10,080 will be taken from ESSA Title IV-A to add all permanent and temporary employees to existing data platforms. $100,000 from ESSA Title IV-A will be allocated to communications. Finally, $50,000 will be set aside for operating, miscellaneous, unexpected, and emergency expenses to be distributed across the entire program (Salesforce, 2025).

Totals. The total five-year investment is: **$26,680,491.94**

- First year (12 schools across 2 wards): $2,219,391.28
- Second year (30 schools across 4 wards): $3,824,918.40
- Third year: (60 schools across 6 wards): $5,601,462.19
- Fourth year (90 schools across 8 wards): $7,455,887.74
- Fifth year (full implementation for all 117 schools across 8 wards): $7,556,332.29

This investment amounts to an average of $190.61 spent per child per year. While not included in the above calculations, this budget may result in marginal cost savings beyond its provisions. Namely, reducing chronic absenteeism protects schools' enrollment-based funding and reduces the long-term costs of remediation when CA is not corrected early. Additionally, there are economic benefits for D.C. in increasing median educational attainment among the future workforce and reducing future strain on social services.

VII. STAKEHOLDER ANALYSIS AND POLITICAL CONSIDERATIONS

Successful implementation requires strategic navigation of D.C.'s political landscape. Using a

power-interest framework (Eden & Ackermann, 1998), the below section identifies critical stakeholders and political dynamics.

A. Stakeholder Analysis

Mayor Bowser and Executive Office. The Mayor holds ultimate authority over DCPS. Her "Every Day Counts!" taskforce demonstrates existing commitment to addressing CA. The proposal's equity focus aligns directly with mayoral priorities. However, competing demands across housing and public safety create fiscal constraints. To secure mayoral support, the framework must emphasize evidence-based equity outcomes and return on investment through protected enrollment funding.

D.C. City Council. Recent legislation, including the 2024 Truancy Reduction for Student Success Act and Councilmember Parker's 2025 Amendment Act, demonstrates Council receptivity to absenteeism interventions. However, members will likely scrutinize multi-year costs and programmatic sustainability. Successful Council engagement requires providing ward-specific data to the Education Committee, particularly highlighting benefits for Wards 5, 7, and 8 where chronic absenteeism rates are highest, while emphasizing coordination of existing resources over purely new spending.

OSSE and Partner Agencies (DHS, CFSA, DHCD, DOH). OSSE would lead coordination and implementation across pillars. The Truancy Reduction pilot demonstrated DHS-DCPS coordination and provided evidence that cross-agency collaboration can reduce CA. However, fragmented data systems and insufficient coordination remain barriers. Implementation requires mayoral directive establishing clear memoranda of understanding, dedicated interagency liaisons, and framing hub participation as advancing each agency's core mission rather than adding burden. The success of Attendance Action Hubs depends on genuine integration, including shared data systems, co-located staff, and aligned performance metrics.

Teachers and Principals. The buy-in of teachers and principals is critical to programmatic success. Implementation strategy must include early Washington Teachers' Union engagement, clarifying that family engagement liaisons support rather than supervise teachers. While teachers may resist additional workload, the Family Engagement Corps is designed to reduce their burden by handling

intensive outreach currently falling on classroom staff. The strategy should also include collaborative pilot school selection factoring in principal readiness, dedicated implementation support, and public recognition of high achieving schools through the Student Attendance Leaderboard.

Parents and Families. Family cooperation is essential for attendance improvement. Shifts toward family-based rather than behavioral causes of absenteeism especially underscore the importance of authentic partnerships with parents. Building trust with families from historically marginalized communities requires three critical elements: pre-implementation community listening sessions that emphasize barrier removal over blame; culturally competent Family Engagement Corps liaisons with strong community ties and relevant language skills; and parent advisory committees with genuine decision-making input, not merely symbolic representation.

B. Political Opportunities and Challenges

Opportunities. The post-pandemic environment creates a convergent "policy window" (Kingdon, 2011) where problem recognition, available solutions, and political will align. D.C.'s consolidated mayoral authority over education, health, housing, and social services enables coordinated action impossible in jurisdictions with fragmented governance. The pillars framework directly addresses documented disparities, targeting students most affected by CA. Critically, the Truancy Reduction pilot's 50% attendance increase (D.C. Department of Human Services, 2025) provides local proof that supportive, service-connected interventions work in D.C.'s specific context, reducing political risk.

Challenges. Implementation faces two interconnected challenge categories. Fiscally, multi-year costs require substantial appropriations amid intense competition from housing, public safety, and infrastructure priorities. Sustainability beyond time-limited federal grants demands demonstrated return on investment, which is particularly challenging given that meaningful CA reductions require multiple years to materialize while electoral cycles create pressure for quick, visible results. Coordination challenges are equally formidable. Fragmented data systems and insufficient agency coordination have historically limited timely interventions despite good intentions (AJE-DC, 2025). The Attendance Action Hubs require genuine integration across agencies with different organizational cultures, priorities,

accountability structures, and technology systems, moving beyond the parallel operations that have characterized past D.C. initiatives (Pressman & Wildavsky, 1973). Additional risks include recruiting qualified Family Engagement Corps liaisons and health professionals in competitive labor markets, technology procurement delays, and the challenge of building authentic trust with families who may be skeptical of government outreach due to a long history of marginalization.

D.C.'s governance structure, documented equity crisis, and demonstrated local success create favorable conditions for transformative action. However, success depends on strategic stakeholder engagement that recognizes each group's distinct interests and concerns, transparent accountability mechanisms that build and maintain trust across communities, and sustained political will to address the complex, interconnected barriers keeping over 32,000 students from consistent school attendance.

VIII. CONCLUSION

CA in Washington, D.C. is not an isolated education problem but a multidimensional equity crisis that undermines students' academic, social, and economic futures. This paper demonstrates that meaningful progress will require the District to move beyond fragmented, school-by-school initiatives and toward a coordinated, data-driven, districtwide strategy. By implementing a comprehensive five-pillar framework, centered on family engagement, early warning systems, community-based action hubs, expanded school-based health care, and transparent accountability, D.C. can tackle the root causes of absenteeism while strengthening trust between schools, families, and public agencies. With mayoral authority, proven evidence from peer jurisdictions, and a clear implementation and funding path, the District has both the tools and the political opportunity to act. By adopting this proposal, D.C. would not only reduce CA but also advance a more equitable education system that supports every child's ability to learn, thrive, and succeed.

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APPENDIX

Figure 1: CA Rates by School Year in D.C. (Data extracted from OSSE, 2024 & 2025)

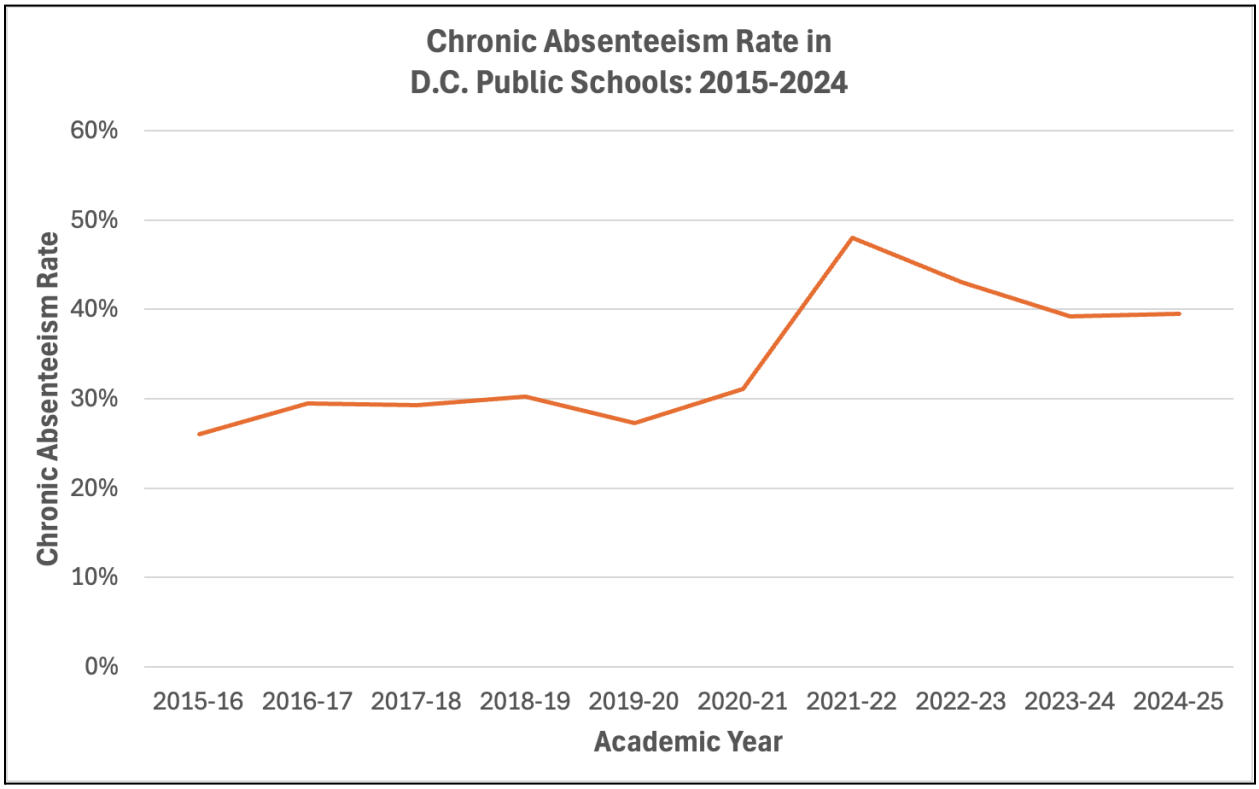

Figure 2: Causal Model for Chronic Absenteeism

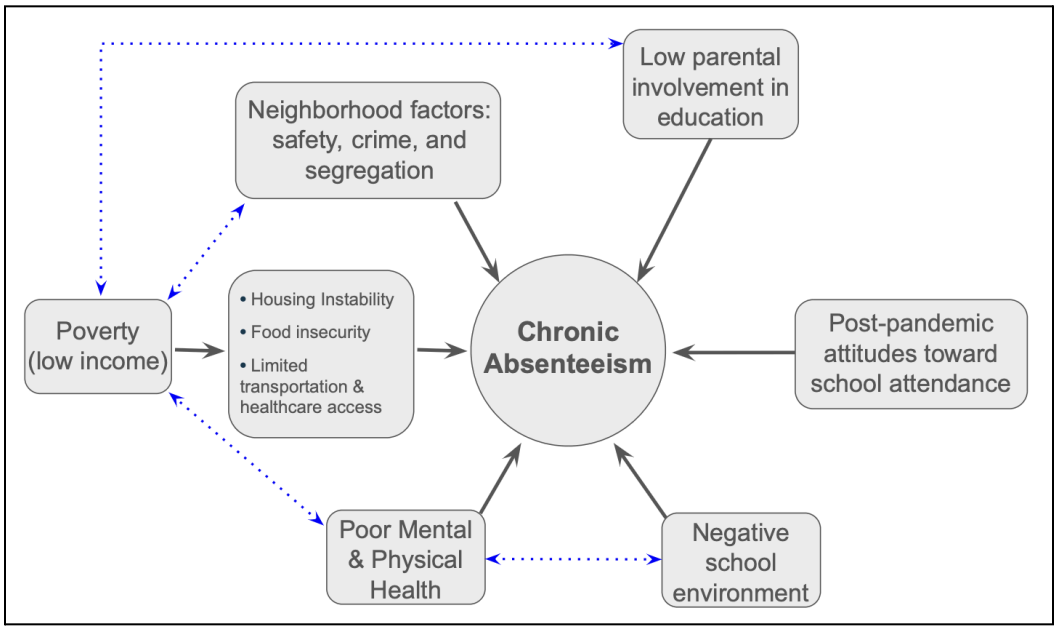

Figure 3: Itemized Budget

Year	Pillar	Line Item	Expense	Source
1	1	Comms	$240.00	ESSA Title I-A
1	1	Staffing	$185,921.18	ESSA Title I-A
1	1	Training	$14,301.63	ESSA Title I-A
1	2	Technology	$60,000.00	ESSA Title I-A
1	2	Training	$14,301.63	ESSA Title II-A
1	3	Comms	$2,400.00	ESSA Title I-A
1	3	Staffing (Perm.)	$185,921.18	TANF
1	3	Staffing (Addl.)	$185,921.18	ESSA Title I-A
1	3	Venue	$24,108.80	TANF
1	4	Staffing	$811,802.36	Medicaid Reimb.
1	4	Supplies	$3,539.06	Medicaid Reimb.
1	4	Training	$28,603.26	ESSA Title IV-A
1	4	Vehicle	$200,000.00	HRSA Grant
1	5	Comms	$500.00	ESSA Title IV-A
1	OEA	Comms	$20,000.00	ESSA Title IV-A
1	OEA	Rainy Day Fund	$10,000.00	ESSA Title I-A
1	OEA	Staffing (Perm.)	$470,571.00	TANF
1	OEA	Staffing (Addl.)	$0.00	ESSA Title I-A
1	OEA	Technology	$1,260.00	ESSA Title IV-A
2	1	Comms	$480.00	ESSA Title I-A
2	1	Staffing	$382,997.63	ESSA Title I-A
2	1	Training	$14,730.68	ESSA Title I-A
2	2	Technology	$150,000.00	ESSA Title I-A
2	2	Training	$7,365.34	ESSA Title II-A
2	3	Comms	$4,800.00	ESSA Title I-A
2	3	Staffing (Perm.)	$382,997.63	TANF
2	3	Staffing (Addl.)	$382,997.63	ESSA Title I-A
2	3	Venue	$48,217.60	TANF
2	4	Staffing	$1,672,312.86	Medicaid Reimb.
2	4	Supplies	$7,078.12	Medicaid Reimb.
2	4	Training	$29,461.36	ESSA Title IV-A
2	4	Vehicle	$200,000.00	HRSA Grant

2	5	Comms	$500.00	ESSA Title IV-A
2	OEA	Comms	$20,000.00	ESSA Title IV-A
2	OEA	Rainy Day Fund	$10,000.00	ESSA Title I-A
2	OEA	Staffing (Perm.)	$494,099.55	TANF
2	OEA	Staffing (Addl.)	$15,200.00	ESSA Title I-A
2	OEA	Technology	$1,680.00	ESSA Title IV-A
3	1	Comms	$720.00	ESSA Title I-A
3	1	Staffing	$591,731.34	ESSA Title I-A
3	1	Training	$15,172.60	ESSA Title I-A
3	2	Technology	$300,000.00	ESSA Title I-A
3	2	Training	$22,758.90	ESSA Title II-A
3	3	Comms	$7,200.00	ESSA Title I-A
3	3	Staffing (Perm.)	$591,731.34	TANF
3	3	Staffing (Addl.)	$591,731.34	ESSA Title I-A
3	3	Venue	$72,326.40	TANF
3	4	Staffing	$2,583,723.37	Medicaid Reimb.
3	4	Supplies	$10,617.18	Medicaid Reimb.
3	4	Training	$30,345.20	ESSA Title IV-A
3	4	Vehicle	$200,000.00	HRSA Grant
3	5	Comms	$500.00	ESSA Title IV-A
3	OEA	Comms	$20,000.00	ESSA Title IV-A
3	OEA	Rainy Day Fund	$10,000.00	ESSA Title I-A
3	OEA	Staffing (Perm.)	$518,804.53	TANF
3	OEA	Staffing (Addl.)	$32,000.00	ESSA Title I-A
3	OEA	Technology	$2,100.00	ESSA Title IV-A
4	1	Comms	$960.00	ESSA Title I-A
4	1	Staffing	$812,644.37	ESSA Title I-A
4	1	Training	$15,627.78	ESSA Title I-A
4	2	Technology	$450,000.00	ESSA Title I-A
4	2	Training	$23,441.66	ESSA Title II-A
4	3	Comms	$9,600.00	ESSA Title I-A
4	3	Staffing (Perm.)	$812,644.37	**ESSA Title IV-A**
4	3	Staffing (Addl.)	$812,644.37	ESSA Title I-A
4	3	Venue	$96,435.20	**ESSA Title I-A**

4	4	Staffing	$3,548,313.43	Medicaid Reimb.
4	4	Supplies	$14,156.24	Medicaid Reimb.
4	4	Training	$31,255.55	ESSA Title IV-A
4	4	Vehicle	$200,000.00	HRSA Grant
4	5	Comms	$500.00	ESSA Title IV-A
4	OEA	Comms	$20,000.00	ESSA Title IV-A
4	OEA	Rainy Day Fund	$10,000.00	ESSA Title I-A
4	OEA	Staffing (Perm.)	$544,744.75	**ESSA Title IV-A**
4	OEA	Staffing (Addl.)	$50,400.00	ESSA Title I-A
4	OEA	Technology	$2,520.00	ESSA Title IV-A
5	1	Comms	$960.00	ESSA Title I-A
5	1	Staffing	$837,023.70	ESSA Title I-A
5	1	Training	$0.00	ESSA Title I-A
5	2	Technology	$585,000.00	ESSA Title I-A
5	2	Training	$24,144.91	ESSA Title II-A
5	3	Comms	$9,600.00	ESSA Title I-A
5	3	Staffing (Perm.)	$837,023.70	ESSA Title IV-A
5	3	Staffing (Addl.)	$837,023.70	ESSA Title I-A
5	3	Venue	$96,435.20	ESSA Title I-A
5	4	Staffing	$3,654,762.83	Medicaid Reimb.
5	4	Supplies	$14,156.24	Medicaid Reimb.
5	4	Training	$0.00	ESSA Title IV-A
5	4	Vehicle	$0.00	HRSA Grant
5	5	Comms	$500.00	ESSA Title IV-A
5	OEA	Comms	$20,000.00	ESSA Title IV-A
5	OEA	Rainy Day Fund	$10,000.00	ESSA Title I-A
5	OEA	Staffing (Perm.)	$571,981.99	ESSA Title IV-A
5	OEA	Staffing (Addl.)	$55,200.00	ESSA Title I-A
5	OEA	Technology	$2,520.00	ESSA Title IV-A
Sbtot.	1		$ 2,873,510.91	
Sbtot.	2		$ 1,637,012.45	
Sbtot.	3		$ 5,991,759.66	
Sbtot.	4		$13,240,127.10	
Sbtot.	5		$ 250,000.00	

Sbtot.	OEA	$ 2,913,081.82	
1	Subtotal	$ 2,219,391.28	
2	Subtotal	$ 3,824,918.40	
3	Subtotal	$ 5,601,462.19	
4	Subtotal	$ 7,455,887.74	
5	Subtotal	$ 7,556,332.29	
Subtotal	I-A Expenditures	$ 7,658,099.54	ESSA Title I-A
Subtotal	II-A Expenditures	$ 92,012.45	ESSA Title II-A
Subtotal	IV-A Expenditures	$ 2,998,640.19	ESSA Title IV-A
Subtotal	TANF Expenditures	$ 2,788,778.03	TANF
Subtotal	MR Expenditures	$12,320,461.70	Medicaid Reimb.
Subtotal	HRSA Expenditures	$ 800,000.00	HRSA Grant
TOTAL BUDGET		**$26,657,991.89**	